833-208-7770
contactus@imaginepediatrics.org
imaginepediatrics.org

Imagine Pediatrics
220 Athens Way, Suite 240
Nashville, TN 37228

June 16, 2025

Re: CMS-0042-NC - Request for Information; Health Technology Ecosystem

Dear Administrator Oz,

On behalf of Imagine Pediatrics, we are pleased to submit comments in response to the Centers for Medicare & Medicaid Services (CMS) Request for Information (RFI) on the Health Technology Ecosystem.

About Imagine Pediatrics

Imagine Pediatrics is a pediatrician-led medical group delivering 24/7 virtual and in-home care to children with special health care needs. We currently serve more than 35,000 children and their families across Texas, Florida, and the Washington D.C. metro area through value-based arrangements with six Medicaid managed care organizations. Our multidisciplinary team of 400+ staff members, approximately 70% of whom are clinical team members, works in partnership with existing providers to deliver expanded access to care that helps children spend more safe days at home.

Since launching care delivery for our first patients in January 2023, we have demonstrated remarkable outcomes:

- We've proven that integrated care can actually cost less, with a 15% Medical Loss Ratio reduction to date. This isn't just innovation for innovation's sake – it's real impact for real families and it has the power to transform the health industry
- 86+ Net Promoter Score from patients/caregivers
- Prevention of nearly 10,000 unnecessary emergency and urgent care visits through timely primary interventions
- 76% of those diverted ED visits required no acute care in the following 30 days

As a tech-enabled medical group serving children with special health care needs who are Medicaid-enrolled, Imagine Pediatrics brings a distinctive vantage point to these technology challenges. Children account for nearly half of Medicaid enrollees – an estimated 37 million. Among them are 14.5 million children with special health care needs, about half of whom depend on Medicaid for their care, but policy has not yet fostered a proliferation of independent provider groups willing to take double-sided risk to deliver on the needs of this population. These achievements for that vulnerable population underscore how effective health

technology infrastructure can be in transforming care delivery with far-reaching impact. To this end, we appreciate CMS actions and the Agency's support for legislation that preserves telehealth access in Medicaid and Medicare, protects and expands telehealth in commercial coverage and safeguards remote prescribing of clinically appropriate controlled substances.

Real-Time Care Coordination Across Fragmented Systems: Our 24/7 fully integrated care model requires us to navigate multiple EHRs, HIEs, and payor systems simultaneously. We see firsthand how the lack of behavioral health data integration and inconsistent provider directories directly impact our ability to prevent unnecessary ED visits and hospitalizations.

Bridging Digital Divides: Serving 35,000 families receiving Medicaid across three states, we understand both the potential and limitations of digital health tools for vulnerable and underserved populations. While 41% of Medicaid members haven't been offered digital health apps (according to a recent BCG report), we've achieved an 86+ NPS patient/caregiver score by designing technology that meets families where they are through intuitive omni-channel digital tools that create a cohesive patient experience across modalities - both virtually and in-home.

Value-Based Care Innovation: Our 15%+ Medical Loss Ratio reduction demonstrates what's possible when technology enables proactive, personalized, coordinated care. However, achieving these results required building custom solutions to work around current technology gaps—investments that many providers cannot afford without stable, long-term incentive structures.

Caregiver-Centered Design: Pursuant to a white paper we recently published, our care model recognizes that engaged caregivers are essential healthcare partners. Every technology recommendation we make is informed by our daily experience supporting families who are coordinating medical, mental health and social care for children with special health care needs across multiple providers and settings. We have convened a group of caregivers – our Patient & Family Advisory Committee – to provide feedback on their experience of care at Imagine Pediatrics, and many examples below draw from this group of passionate and engaged caregivers.

This operational experience in managing real-time care delivery 24/7 across both urban and rural geographies while navigating technological barriers provides us with unique insights into which policy changes would have the most meaningful impact on care delivery and outcomes.

1. Behavioral Health Data Integration Remains a Critical Gap

The Challenge: Despite advances in interoperability, behavioral health data remains largely inaccessible. Many behavioral health facilities do not utilize certified EHRs and are therefore absent from health information networks. In addition, many large health systems that perform behavioral health care keep such data carved entirely out of the HIE networks to which they are otherwise connected for the risk of inappropriate disclosure. This forces families caring for children with complex needs to repeatedly share their child's behavioral health history with each new provider. Expanding TEFCA or other HIEs to include behavioral health is critical to advancing provider access as detailed in PR-6 and PR-7.

Our Experience: As noted in our introduction, our 24/7 care model requires real-time coordination across fragmented systems to bridge gaps in access and care. This challenge is most acute with behavioral health data. Recently, a 13-year-old patient in the foster care system with PTSD and mood disorders was discharged from a behavioral health facility while transitioning between foster placements. Despite our care coordination efforts, we only learned of the discharge through live discussion with the new foster facility. The discharge medication regimen was never communicated to the new caregivers, and our team discovered the patient was on duplicate doses of one medication and needed an urgent refill for another. Our Behavioral Health and Pharmacy teams spent hours piecing together this critical information—work that could have been avoided if the facility were connected to an HIE. This gap in real-time data sharing directly impacts our ability to prevent unnecessary ED visits and potential patient harm.

Recommendations:

- **Provide incentives for behavioral health facilities to adopt certified EHR technology**: Leverage existing authorities to incentivize EHR adoption through:
    - Medicaid: Enhanced managed care contract requirements directing plans to provide technology grants; 1115 demonstration waivers for health IT infrastructure; CCBHC expansion grants with IT requirements; 90/10 administrative matching funds for HIE connectivity
    - Medicare: Use Innovation Center (CMMI) demonstrations to test behavioral health IT adoption models; incorporate IT requirements into Medicare Shared Savings Program behavioral health integration tracks; leverage Conditions of Participation updates to require data sharing capabilities
    - Cross-program: Align Medicare and Medicaid requirements through integrated care models for dual-eligibles; use existing SAMHSA block grants with enhanced IT requirements

- **Clarify that sharing behavioral health information for treatment purposes is permitted and encouraged under existing privacy laws:** Provide clearer guidance on how existing privacy laws, including 42 CFR Part 2 and HIPAA, can be navigated to facilitate appropriate sharing of behavioral health information for treatment purposes through HIEs [PR-3(d): Privacy implications]

2. Text Messaging as an Essential Digital Health Tool

The Challenge: Despite strong evidence that patients and caregivers readily adopt and benefit from SMS-based care, texting remains under-recognized as a digital tool that supports care coordination. Requiring an app download introduces significant friction: many phones have limited storage or data plans, and unreliable broadband can make large downloads and updates impractical.

Our Experience: Our Patient & Family Advisory Committee (PFAC) consistently identifies text messaging as a critical communication preference. Families love to interact with us digitally, and SMS is their preferred method of communication with our teams. We also often hear that families struggle with app storage on their phones, remembering login credentials for phones that don't support biometrics, occasionally older caregivers who struggle with the app download process, and some households that are not able to complete downloads and updates due to poor connectivity.

SMS eliminates these barriers entirely - no downloads, no passwords, no storage requirements. This accessibility is why texting has proven so effective for appointment reminders, quality gap closure nudges, care coordination, and disease management programs - including studies showing a 14.6% reduction in no-show appointment rates when patients were texted appointment reminders vs. those who were not texted (Link), and 35% higher rates of primary care visits at 12-month follow up for children with asthma who received in-person and text-based educational programming vs. those who did not (Link).

Recommendations:

- **Issue non-binding guidance recognizing SMS as a qualified digital health tool:** CMS should clarify that text messaging programs qualify as digital health products when used for care-related purposes, including appointment reminders and scheduling, quality gap closure, care plan education and adherence, and disease management programs [PC-6: Features for accessibility]

This recognition would give providers confidence to invest in SMS-based care programs that meet families where they are, reducing barriers to engagement for the millions of Medicaid-covered children and their caregivers who need accessible, evidence-based digital touchpoints.

3. National Digital Identity Infrastructure Would Transform Patient Access

The Challenge: Families caring for children with special health care needs must manage dozens of provider portal credentials. As a result, we are strongly supportive of advancing PC-14: Digital identity challenges from the RFI.

Our Experience: Our success in bridging digital divides comes from meeting families where they are. However, even our simplified approach faces barriers. In the case of pediatrics (and more specifically, care for foster children), a national patient identity structure would ease the burden of children moving between caregivers and medical consenters. Our caregivers frequently must reset digital health application account credentials, particularly in highly sensitive moments such as acute presentations to hospitals.

For example, a foster child in our care recently transferred to a new foster setting. The prior caregiver was unable to identify the new foster placement. The prior caregiver lost access to our app, as it was no longer appropriate for them to have access to the child's PHI. So we had to regularly follow up with the foster system to 'find' the patient who was enrolled in our care.

A national identity inclusive of consenter responsibilities would have prevented delays in care and allowed us as a longitudinal care team to follow the patient across contexts. The current fragmented identity system forces families to choose between security (unique passwords) and access (remembering credentials during crisis moments). This directly contradicts our caregiver-centered design principle and CMS' own goals in the RFI for caregivers that technology should reduce, not increase, family burden.

Recommendations:

- **Support adoption of standards like OpenID Connect across all CMS programs:** Lead by example and require Medicare, Medicaid, and Exchange systems to function as identity providers, enabling single sign-on across federal health programs. Implement NIST 800-63-3 IAL2/AAL2 standards allowing patients to use verified credentials (CLEAR, ID.me, Login.gov) across all CMS systems [PC-14(a): Implementation approach]

- **Establish national digital identity credentials as an option (not requirement) for patients:** Require payers and providers receiving federal funds to use digital identity for health information exchange and patient portal workflows [PC-14: Digital identity]
    - Organizations should accept the aforementioned credentials for health information exchange by 2026 via TEFCA or an equivalent network
    - Organizations should allow for account creation, password reset, and other identity-sensitive moments using digital identity while maintaining traditional authentication options by 2027
- **Design identity solutions specifically tested with Medicaid populations and caregivers managing multiple dependents**: Create roadmap and glidepath for identity providers to support harder edge cases and move towards a digital-only future [TD-3: Implementation considerations]
    - Authorized representative/guardian access with role-based permissions
    - Mobile-first authentication (no desktop/scanner requirements)
    - Multiple linked profiles under one caregiver account
    - Low-literacy interface options and video-based verification alternatives
    - Integration with state Medicaid eligibility systems for auto-verification
- **Ensure identity solutions work across state lines:** Establish federal identity federation requirements allowing credentials issued in one state to access records nationwide. Prohibit state-specific identity requirements that block interstate care coordination. Create reciprocity agreements similar to driver's license compacts [PC-14(e): Cross-state functionality]
- **Maintain strong privacy protections while improving access**: Implement privacy-preserving technologies **[PC-14(c): Privacy concerns]**
    - Selective disclosure (share only necessary identity attributes)
    - Consent management APIs tracking data sharing preferences
    - Audit logs accessible to patients showing who accessed their identity
    - Prohibition on selling or sharing identity verification data
    - Right to delete identity credentials while maintaining care access

4. Provider Directory Accuracy Impacts Care Access

The Challenge: Inaccurate provider directories create barriers to care, particularly for specialty services. Our care coordinators report that provider directories are more often wrong than right, impacting our ability to connect families to timely specialist care.

Our Experience: Our caregiver-centered design philosophy means we measure success by how much time and stress we save families. Yet, provider directory inaccuracy undermines these efforts daily.

We face systemic provider mapping challenges. Recently, we attempted to coordinate care with a multi-specialty practice in Florida, where we believed we shared 100 patients based on patient-provider mapping from our MCO partner. After extensive manual reconciliation, we discovered we actually shared 350 patients. Without accurate mapping of providers to their practice affiliations, we had no way of knowing the overlap was this large—a missed opportunity to scalably approach patients jointly with the practice and ensure coordinated care across PCPs and digital health providers.

Most critically, directory failures directly harm the continuity of care. We recently discovered that a primary care provider had moved out of the area months earlier, but this change was never reflected in any directory. Several of our patients who weren't seeing her regularly were lost to care entirely. If we had been made aware of the provider's departure through up-to-date listings, our model is specifically built to intercept these transitions and ensure continuity by bridging patients to new providers. Instead, these vulnerable children experienced gaps in care that could have been prevented.

Additionally, the existing Medicaid provider enrollment landscape presents substantial inefficiencies. Healthcare providers must navigate 50+ different state enrollment systems, each with unique requirements, timelines, and documentation standards. This fragmentation creates several critical issues:

- Providers face enormous administrative costs in maintaining separate enrollments across multiple states, particularly affecting multi-state healthcare systems and telemedicine providers.
- The redundant credentialing processes delay patient access to care as providers await approval in each jurisdiction.
- Data inconsistencies across state systems create gaps in provider directories, making it difficult for patients to locate participating providers.
- States bear significant costs in maintaining separate enrollment infrastructures, and the lack of standardization complicates oversight and fraud prevention efforts.

This wide variety of directory failures, from individual availability to practice affiliations to provider departures, underpins the breakdowns in care we work every day to prevent.

Recommendations:

- **Establish a national provider directory with FHIR endpoints:** This unified directory should be publicly accessible, machine-readable, and serve as the authoritative source for provider information across all payers and healthcare systems [TD-5: Nationwide directory specifications]
    - Provider status changes (departures, moves, practice changes)
    - Provider-to-practice organizational mappings
    - Accurate specialty taxonomy with standardized service descriptions
    - Real-time Medicaid acceptance status
    - Hours of operation (including 24/7 availability)
    - New patient acceptance status by insurance type updated in real-time
- **Mirror Medicare's PECOS to create a Universal Medicaid Provider Enrollment System (UMPES):** Establish a unified national Medicaid provider enrollment system to reduce administrative burden, improve data accuracy, and enhance patient access to care. [TD-5]
    - Replace the current fragmented state-by-state enrollment processes with a single, streamlined system
    - Provide each Medicaid provider with a single, national provider identification number linked to comprehensive provider data (stated above)
    - Maintain real-time provider directory information accessible to all state Medicaid programs, beneficiaries, and managed care organizations ability to set specific participation requirements
    - Establish federal standards for core enrollment data elements while allowing states flexibility for program-specific requirements
    - Incorporate automated updates for provider information changes, reducing the current manual burden of notifying multiple state systems
- **Require real-time attestation of provider availability:** Move beyond quarterly updates to real-time status changes for new patient availability, ensuring families know immediately if a provider can see them [TD-5]
- **Standardize specialty and service taxonomies:** Replace confusing taxonomy codes with plain-language specialty and service descriptions that families can understand [VB-15: Directory elements for care coordination]
- **Enable bidirectional feedback mechanisms:** Allow providers to update and verify directory information in real-time [TD-5]

- **Include specialty pediatric providers and therapy services critical for children with special health care needs, such as providers of Applied Behavior Analysis and Durable Medical Equipment** [VB-15: Directory elements for care coordination]

5. Variability in Quality Measure Data Reporting and Measurement Hinders Innovation

The Challenge: While quality requirements are essential for ensuring whole-population care, today's fragmented quality measure stewardship creates significant operational burdens for innovative at-risk providers. Currently, both payers and providers must independently calculate (or pay third parties to calculate) patient- and population-level quality statuses over time. This duplicative system leads to year-end reconciliations that leave both parties uncertain about true performance, while score calculations that require consolidation of data across many stakeholders and sources make it nearly impossible to maintain a real-time view of the quality gap in care closure in order to ensure proper and personalized care.

Our Experience: Our value-based care innovation—achieving 15% medical cost ratio reduction—requires us to navigate not just contract variability but disagreements about quality performance across our six Medicaid Managed Care partners. Each plan uses different processes and data sources to capture quality metric information for the same HEDIS measures, forcing us to maintain six parallel quality submission and tracking systems. Additionally, the lack of timely claim and encounter information across a fragmented care delivery ecosystem creates lagging information related to which patients are still in need of care gap closure.

Our care teams huddle each morning to plan their visits for the day. One key component of the daily huddle is preparing to assist with the closure of open quality gaps for patients on the schedule. Our teams review our own internally calculated measure status, but then also must check payor portals and other HIE-based external clinical documentation to gain the best picture of “true” gap status. Mismatches between sources create frustration for care teams and, more importantly, abrasion for caregivers. For example, we may ask a caregiver to see their PCP for their well-child visit or to complete a social needs screener, when in fact that visit or screener has already been completed for the measurement period. These barriers erode trust in the healthcare system.

In order to provide the highest quality preventive care, we encourage CMS to address today's system of duplicative calculations and divergent data submission criteria between payers and

providers via a unified reporting mechanism viewable by all relevant stakeholders. We'd be excited to contribute data to such a mechanism.

Recommendations:

- **Create a unified quality registry accessible to all stakeholders:** Implement a real-time registry where every provider can see a patient's current preventive care status, enabling gap closure at every encounter [PR-8(c): Registry requirements for real-time data]
- **Create real-time FHIR-based quality APIs to augment existing flat file gaps list sharing:** Transition from static file exchanges to dynamic, standardized APIs that provide current quality gap information [TD-9(e): Bulk FHIR for quality]
- **Implement the DaVinci Data Exchange for Quality Measures:** Incentivize use of this FHIR-based standard to ensure consistent data formatting and real-time updates across all stakeholders [TD-11: Standardized export capabilities]
- **Provide financial incentives for unified reporting:** Offer enhanced payments to organizations that contribute real-time quality data to the unified registry [VB-1: Incentives for digital adoption]

6. Alternative Payment Models Require Clear Regulatory Boundaries for Technology Innovation

The Challenge: Current regulations create uncertainty about when clinical technologies cross into population health management decision-making that could be construed as benefiting some patients over others based on insights from analytics that rely on incomplete underlying data sets. This regulatory ambiguity chills innovation, as APM participants fear that implementing capabilities such as AI-driven risk stratification or population health analytics could trigger additional compliance requirements or litigation. With recent scrutiny and lawsuits against payers and providers across the industry for various uses of AI, providers in value-based arrangements need clear regulatory boundaries that provide clarity on clinical care delivery informed by innovative technologies.

Our Experience: Our value-based care model enables our clinicians to identify children at risk of hospitalization and intervene proactively. For example, our algorithms flag when a child with special health care needs shows diagnostic signs of clinical deterioration, allowing us to surge our efforts to that child through proactive outreach. We have structured AI tools that assess rising risk to ensure the right care is delivered at the right time to the right patient, across our whole population. AI tools have the opportunity to identify who needs *more* support, not less.

Recommendations:

- **Explicitly clarify that clinical population health management informed by analytics and innovative technologies is appropriate in APM contexts:** CMS should clarify that AI and analytics tools used for risk stratification, care planning, and population health management constitute clinical care when used by providers in value-based arrangements [VB-7: Reducing complexity while preserving flexibility]
- **Create a "safe harbor" for APM technology innovations:** Establish clear criteria that exempt clinical decision support tools from utilization management regulations when they:
    - Are used to enhance care delivery rather than deny services
    - Support proactive interventions rather than retrospective denials
    - Are transparent to patients and their care teams
- **Provide technical assistance for APM participants:** Help organizations understand the boundaries between permissible clinical innovation and perceived patient discrimination activities [TD-1: Stimulating innovation]
- **Support a standard healthcare-specific framework for developing and using AI models**, similar to HITRUST and SOC for information security, that would provide stakeholders with confidence that business associates are using AI responsibly, safely, and with clinical accuracy.

This regulatory clarity would unleash innovation in value-based care, allowing more organizations to deploy the technologies that have enabled our success in keeping vulnerable children healthy at home.

7. General Interoperability Considerations

The Challenge: Despite significant progress in health data exchange, fundamental interoperability gaps persist that impact daily care delivery. CCDA documents often arrive with inconsistent formatting and missing critical data elements like upcoming appointments. Cross-state care coordination remains fragmented, with state-based HIE models creating barriers for patients seeking specialty care across state lines. Additionally, evolving privacy concerns post-Dobbs have reignited debates about opt-in versus opt-out consent models, potentially restricting data availability for care coordination.

Our Experience: As a provider serving children with special health care needs across Texas, Florida, and the Washington D.C. metro area (with plans to rapidly scale to new states), we

encounter these interoperability challenges daily. When receiving CCDAs from various health systems, we find that critical information is often missing or inconsistently formatted. We are proud to offer upcoming appointment reminders to families under our care, including reminders about appointments with external specialists or therapists. Children with special health care needs routinely see multiple providers, placing a large burden on families to take time away from work and children out of school. Value-based providers like us could support families by aligning frequent appointments to the same day and ensure coordinated transportation, but the burden often falls on the family to accomplish this. Without complete information about upcoming external appointments, these capabilities remain nascent and fragmented.

Our appointment reminder program is just one of many capabilities that would be enhanced by deeper and clearer flow of patient data across contexts. Members of our PFAC maintain physical 'care binders' as the best way to understand their child's full health picture, as digital medical records do not consolidate well across the many specialists our patients encounter throughout their journey of care. A member of our PFAC recently told us: "Care coordination should not depend solely on caregiver capacity. We need tools that treat families as partners — not data entry clerks — and shift part of the system's responsibility back to those being paid to deliver care. A collaborative care binder is not just a tech upgrade; it's a lifeline for caregivers and a critical piece of the interoperability puzzle."

Each time a child encounters a new provider, caregivers must re-explain years of surgeries, therapies, and medication changes. This storytelling is time consuming, and with particularly complex pediatric patients, reliving past medical escalations can be traumatizing for caregivers. A member of our PFAC who cares for two children with severe behavioral health issues tells us: "It's very frustrating to continue to have to repeat the same information. Not only that, it's triggering. I have to have my boys with me and my boys have to sit with me and revisit the same traumas over and over. Coming out of these appointments in the past, the boys have had episodes specifically because we have had to revisit the past over and over and not just focus on healing."

Despite requirements in the ASTP's certified health IT program via the §170.315(b)(1) Transitions of care criteria that EHRs should support USCDI v1 in their C-CDA documents and USCDI v3 in their documents after December 31, 2025. Our experience has been that this is not the case across providers exchanging information for transitions of care on major networks - they are consistently lacking in providing the full expected USCDI dataset.

The state-based HIE model particularly impacts our most vulnerable patients. Children with rare conditions often need to see specialists in neighboring states, but state HIE boundaries create artificial barriers to data sharing. We've found TEFCA and Carequality networks more effective for cross-state coordination, as they operate on consistent technical standards and governance frameworks rather than the patchwork of state-specific rules and extremely variable data capabilities.

Recommendations:

- **Enforce USCDI completeness for CCDAs on national networks:** Make a requirement of TEFCA, Carequality, and other national networks that all CCDAs transmitted through them include all applicable USCDI data elements, with technical validation at the point of transmission [TD-7: USCDI coverage and compliance]
- **Expand CCDA content requirements:** Mandate inclusion of upcoming appointments, care team contact information, and active care plans in all CCDA transmissions [TD-7: USCDI limitations and improvements]
- **Prioritize national frameworks over state-based models:** Support TEFCA and Carequality as primary vehicles for cross-state data exchange, reducing reliance on state HIE boundaries that don't align with patient care patterns [PC-10: TEFCA advancing patient access]
- **Maintain opt-out consent models for treatment purposes:** Preserve the ability to share data for treatment, payment, and operations without requiring explicit opt-in consent, which creates barriers to timely care coordination [PC-10]
- **Address post-Dobbs privacy concerns without compromising care:** Develop clear guidance that protects sensitive health information while ensuring providers can access complete medical histories for treatment decisions

These improvements would enable seamless care coordination across state lines and care settings, ensuring that children with special health care needs receive timely, informed care regardless of where they seek treatment.

Conclusion

Imagine Pediatrics' experience demonstrates that when technology barriers are removed and caregivers are properly supported, innovative care models can dramatically improve outcomes

while reducing costs. We stand ready to partner with CMS in building a health technology ecosystem that truly serves all Americans, especially children with special healthcare needs.

We appreciate the opportunity to comment and look forward to continued collaboration in advancing health technology infrastructure.

Sincerely,

George Boghos
Chief Executive Officer

Taylor Beery
Co-founder, Chief Innovation & Administrative Officer